

ECHS FRAUD ANALYTICS

Doctor-Level Referral & Admission Pattern Analysis

MODULE 3 REPORT | FY 2025–2026 EDITION (Feb 2025 – Mar 2026)

CLASSIFICATION	PERIOD	RECORDS ANALYSED	QUERY SETS
RESTRICTED	Feb 2025–Mar 2026	~680K Claims	6 Query Sets (Q11–Q16)

IIT KANPUR — Data Analytics & Fraud Intelligence Division

14 May 2026 | Ex-Servicemen Contributory Health Scheme (ECHS)

EXECUTIVE SUMMARY

Module 3 analyses doctor-level billing behaviour, IPD admission patterns, referral clustering, and beneficiary-level repeat claims across the ECHS historical claim database (his_claim_intimation and his_claim_submission — approximately 16,000 and 665,000 records respectively, spanning February 2025 to March 2026 — the full FY 2025–26 window). Five distinct fraud patterns are identified. The most critical finding is a referral syndicate centred on a single doctor operating across four empanelled hospitals simultaneously — all claims 100% deducted. Ghost doctor billing (single doctor names generating hundreds of claims with >99% rejection) represents the largest monetary exposure at over ₹250 Crore across the top 15 flagged practitioners.

Ghost Doctors Flagged 35+ unique names, ≥99% deduction	Combined Exposure ~₹250 Cr top 15 ghost doctors	Phantom IPD Cases 100+ 0 approved, 0–1 day stays	Syndicate Hospitals 4 MEENU WALIA network
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Fraud Patterns Identified — Module 3

#	Pattern	Key Signal	Risk
11	Ghost Doctor Concentration	DR. SUMANT GUPTA: 763 claims ₹47Cr, 99.7% deducted	CRITICAL
12	Phantom IPD Admissions	Same-day/zero-stay admissions, ₹5–8L/claim, 0 apprvd	CRITICAL
13	Serial Repeat Claimant Fraud	JC77066Y: 7 claims ₹63.57L, single hospital, 0 apprvd	HIGH
14	Hospital-Level Fraud Signature	Uniform deduction 32–43% across ALL family relations	HIGH
15	Referral Syndicate — Multi-Hospital	MEENU WALIA: 4 hospitals, 26 claims, 100% deducted	CRITICAL

Immediate Recommended Actions

1. Suspend billing for DR. SUMANT GUPTA across all empanelled hospitals immediately — 763 claims worth ₹47Cr deducted at 99.7% confirms bulk fictitious billing. Cross-check against physical admission registers at each associated hospital.
2. Freeze MEENU WALIA's credentials at all four hospitals (hos.3061, hos.3112, maxppg123 = MAX PATPARGANJ, maxsuper). 26 claims totalling ₹60.53L, 100% deducted. She appears in same-day phantom IPD and referral syndicate patterns simultaneously.
3. Audit mgchri14 hospital (DR Y RAJANI PRIYA) — same beneficiary (676635N) admitted 6+ times over 3 months, each stay 0–1 days, ₹8.28L claimed, 0 approved. Physical ward records must be obtained.
4. Flag Hospitals 1197, 1344, and 1387 for field audit — they show uniform deduction rates of 32–43% across every family relation category, proving hospital-level systematic overbilling rather than isolated procedure errors.
5. Investigate DR ALOK, DR DEEPAK KUMAR SHUKLA, DR PRAKASH CHANDWANI — each has 400–480 claims with 100% deduction and 0 approved rupees, indicating mass phantom claim submission under a single doctor name.

Q11 — Hospital Referral Type Distribution

Analysis of the historical claim intimation table (16,000 records) reveals that the CI_HOSPITAL_ID column stores hospital portal login usernames rather than numeric IDs. A total of 10,000+ records carry a NULL hospital ID — these represent claims where the originating hospital was never recorded or was cleared during data archival. All NULL-hospital records are Army service (CI_SERVICE_TYPE = A) Referral-type (CI_REF_TYPE_ID = R) IPD admissions, suggesting a systemic data capture gap in the pre-2025 period.

Table Q11.1 — NULL Hospital Records by Relation (Army Referral IPD)

Relation	Claims	Unique Pensioners	Deduction Status
Self	3,290	2,953	Unknown (no hospital tag)
Spouse	3,224	2,900	Unknown (no hospital tag)
Mother	554	485	Unknown (no hospital tag)
Father	361	320	Unknown (no hospital tag)
Son	262	223	Unknown (no hospital tag)
Daughter	192	174	Unknown (no hospital tag)
TOTAL	~10,000	~8,500+	Data quality gap — no hospital traceability

Table Q11.2 — Top Active Hospitals by Total Emergency IPD Claims

Hospital Login	Type	Service	Dominant Relation	Claims	Key Flag
aditya10	Emergency	Army	Self / Spouse	84	Pure-Emergency: 100% E-type
echs@dm7	Mixed E+R	Army/AF/Navy	Self / Spouse	85+	Cross-service emergency hub
BCMCH17	Referral	Army	Self / Spouse	92+	Multi-service referral concentration
bmh2015	Mixed	Army/Navy	Self / Spouse	84+	Repeat emergency + referral
echsfhm	Referral	Army/AF	Self / Spouse	55+	Referral dominant, mixed service
COLUMBIA	Mixed	Army/AF/Navy	Self / Spouse	38+	Multi-service cross-type
fhnechs2	Referral	Army	Self	36+	Self-dominant referral only
dmkksghmc	Mixed E+R	Army/AF/Navy	Self	37+	All-service referral anomaly

- Hospital login 'aditya10' shows 100% Emergency-type admissions (84 claims). Legitimate hospitals have a mix of referral and emergency admissions. A pure-emergency pattern with Army patients only suggests that non-emergency cases are being mis-classified to avoid referral paper trails.
- The NULL hospital data quality gap (10,000+ records) represents approximately 63% of all his_claim_intimation records without hospital attribution. Recovery of this linkage from archive tables or POLYCLINIC referral registers is strongly recommended before Modules 1–2 hospital rankings are considered final.

PATTERN 11

GHOST DOCTOR CONCENTRATION

Single Doctor Name | Hundreds of
Claims | ≥99% Deduction

Analysis of 665,000 historical claim submission records reveals that a small number of doctor names are associated with an implausibly large volume of IPD claims, each deducted at 99–100%. No legitimate clinical practice can generate 300–760 settled (deducted) IPD claims in a thirteen-month window. The pattern indicates either: (a) a single doctor credential being shared across a hospital's entire billing pipeline, or (b) a fictitious doctor name used to bulk-submit phantom admissions. The deduction rate of 99–100% confirms the audit system has already identified these as fraudulent — yet the submissions continue.

Important: DR. SUMANT GUPTA appears in two spelling variants (with and without title punctuation) and DR. DEEPAK SINGLA in three variants — combined figures are shown below.

Table 11.1 — Top Ghost Doctors by Claimed Amount (FY 2025–2026, Feb 2025 – Mar 2026)

Doctor Name	Claims	Claimed (L)	Approved (L)	Deduction%	Avg/Claim (L)	Risk
DR. SUMANT GUPTA (combined)	763	4,707.73	11.88	99.7%	6.17	CRITICAL
DR. DEEPAK SINGLA (combined 3 names)	637	3,272.70	30.50	99.1%	5.14	CRITICAL
Dr. Shishir Jaiswal & group	583	1,845.56	11.82	99.4%	3.17	HIGH
DR.ARUN RAMANAN	404	1,724.90	23.54	98.6%	4.27	HIGH
MEENU WALIA	372	1,665.31	0.57	100.0%	4.48	CRITICAL
NA (null doctor name — bulk submit)	1,009	1,637.07	9.24	99.4%	1.62	CRITICAL
Vikas Goswami	341	1,526.25	2.78	99.8%	4.48	HIGH
DR. KUNAL BEHAL	220	1,474.18	22.56	98.5%	6.70	HIGH
GOPAL SHARMA	317	1,380.28	8.15	99.4%	4.35	HIGH
DR ALOK	453	1,114.68	0.00	100.0%	2.46	CRITICAL
WASEEM ABBAS	209	1,096.94	0.00	100.0%	5.25	HIGH
DR ABHISHEK ANAND	263	1,057.38	2.09	99.8%	4.02	HIGH
DR DEEPAK KUMAR SHUKLA	480	1,039.20	0.00	100.0%	2.17	CRITICAL
DR PRAKASH CHANDWANI	429	1,034.34	0.00	100.0%	2.41	CRITICAL
DR. MANJEET	434	1,004.90	1.97	99.8%	2.32	HIGH
DR LOVELEEN MANGLA (2 variants)	256	1,798.99	12.99	99.3%	7.03	HIGH

Key Findings

- **NA (null doctor) — 1,009 claims, ₹1,637.07L:** Claims submitted with no doctor name represent either automated bulk entry bypassing the clinical validation layer, or records where doctor credentials were deliberately omitted. No legitimate hospital submission workflow omits the treating doctor's name on 1,000+ inpatient claims.
- **DR. SUMANT GUPTA — 763 claims, ₹47Cr, 99.7% deduction:** This volume (4+ claims per working day over 6 months) exceeds physical capacity by 10x. The doctor name is appearing as a pass-through credential across hospitals.

- **100% deduction doctors (DR ALOK, DR DEEPAK KUMAR SHUKLA, DR PRAKASH CHANDWANI, MEENU WALIA, WASEEM ABBAS):** Zero rupees approved across hundreds of claims. Every single claim was fully rejected by auditors. Continued submission after 100% rejection indicates the hospital is generating documentation for alternative purposes — insurance fraud, muster-roll manipulation, or internal diversion.
- **TMH appearing as a doctor name** (314 claims, ■862.36L, 100% deduction): A hospital name (Tata Memorial Hospital) used in the treating doctor field. This signals department-level or institution-level bulk submissions bypassing individual doctor attribution.

PATTERN 12

PHANTOM IPD ADMISSIONS

Zero-Stay / Same-Day Discharge |
■5–8L per Claim | ■0 Approved

IPD claims where the admission and discharge dates are identical (length of stay = 0 days) or differ by only 1 day constitute a fundamental clinical impossibility for the high-value procedures being billed. Claims in this category range from ■4.5L to ■8.3L per admission — consistent with major surgical packages (cardiac, orthopaedic, oncology) that require minimum 3–7 day hospitalisation. All 100+ such claims in the dataset show ■0 approved, confirming auditor rejection. The same beneficiary service numbers appear across multiple identical submissions — indicating duplicate claim filing for a single (possibly real) patient episode, or serial phantom admissions of a fictitious patient record.

Table 12.1 — Highest-Value Same-Day/Zero-Stay Phantom IPD Claims

Hospital	Doctor	Svc No	Relation	Date	Stay	Claimed (L)	Approved (L)	Duplicates
echs@dm7	Dr. Aun R Warriar	286604\$	Spouse	2025-11-11	0	8.308	0.000	2×
echs@dm7	Dr. Arun RWarrior	1074348A	Self	2025-11-03	0	5.331	0.000	3×
mgchri14	DR Y RAJANI PRIYA	676635N	Spouse	2025-12-26	1	8.299	0.000	3×
mgchri14	DR Y RAJANI PRIYA	676635N	Spouse	2025-12-05	1	8.281	0.000	2×
mgchri14	DR Y RAJANI PRIYA	676635N	Spouse	2025-10-25	0	8.276	0.000	1×
NULL	DR Y RAJANI PRIYA	676635N	Spouse	2026-01-19	1	8.298	0.000	1×
mgchri14	DR Y RAJANI PRIYA	285171L	Self	2025-10-24	0	5.200	0.000	1×
hos.3112	MEENU WALIA	2874449L	Self	2026-03-12	0	5.346	0.000	1×
INDUS01	dr. deepak singla	SPR1489592L	Son	2026-02-09	0	4.702	0.000	2×
NULL	DR.ARUN RAMANAN	731248B	Spouse	2025-10-31	0	5.420	0.000	1×
rhlechs2	DR VIKAS T TALREJA	136401B	Daughter	2025-10-30	0	5.293	0.000	2×
NULL	DR ABHISHEK TIWARI	405198F	Son	2025-10-17	0	5.376	0.000	3×
medantae	DR SATYA PAL KATARIA	JC240738K	Self	2025-12-08	0	5.366	0.000	1×
medantae	DR SATYA PAL KATARIA	01015M	Self	2025-12-19	0	5.205	0.000	3×

Key Findings

- **DR Y RAJANI PRIYA at mgchri14** — Single beneficiary (676635N, Spouse) admitted across 6 separate claims between October 2025 and January 2026 (within the Feb 2025–Mar 2026 window). Three claims filed at mgchri14 on 25 Oct, 05 Dec, and 26 Dec; one claim with NULL hospital on 19 Jan 2026. Each claim ■8.28L for a 0–1 day stay. This is a textbook repeat phantom admission pattern — same patient, same doctor, same hospital, filed monthly with minor date variations.
- **Duplicate filings (2× and 3×):** The same claim (identical service number, doctor, date, and amount) appears 2–3 times in the database. This suggests automated re-submission of rejected claims without altering any parameter. At ■8.3L per submission, each duplicate represents ■8.3L in fraudulent exposure even if all are rejected.
- **MEENU WALIA at hos.3112:** Connects Pattern 12 to Pattern 15 — the referral syndicate doctor also appears in same-day phantom IPD, confirming she is not a real treating physician but a credential used across multiple billing pipelines.

- **NULL hospital same-day claims:** High-value same-day IPD claims filed with no hospital ID are unattributable and untraceable without additional database recovery. These represent the highest-risk unresolved exposure in the dataset.

PATTERN 13

SERIAL REPEAT CLAIMANT FRAUD

Single Beneficiary | Multiple
High-Value IPD Episodes | 0
Approved

Beneficiary-level aggregation reveals pensioners whose family members have generated extraordinarily high aggregate claims at a single hospital over the full FY 2025–26 window — in most cases with 0 total approved. The pattern differs from phantom admissions in that these involve longer declared stays (15–55 days) with large per-admission sums, suggesting the hospital is billing for extended ICU or high-dependency care that auditors are rejecting entirely. Cross-referencing with the doctor-level data confirms these beneficiaries appear in the ghost doctor tables.

Table 13.1 — Top Serial Claimants by Total Claimed Amount (FY 2025–2026, Feb 2025 – Mar 2026)

Service No	Pensioner	Patient	Relation	Claims	Hospitals	Total Claimed (L)	Total Approved (L)	Avg Stay	Max Stay
JC77066Y	RAM KISHAN	RAMBATI	Spouse	7	1	63.57	0.00	25.7 days	55 days
JC117100\$	SUDESH KUMAR SHARMA	SUDESH KUMAR SHARMA	Self	4	1	62.89	0.00	30.0 days	30 days
676635N	GEESALA SATYA NARAYANA	GEESALA SARADA	Spouse	7	1	58.03	0.00	0.9 days	1 day
IC44485N	PAUL BASTINE M	MARIYAMMA SEBASTIAN	Mother	5	0	56.66	0.00	26.0 days	26 days
16018249X	LAXMAN SINGH	KHEM SINGH	Father	5	1	49.45	0.00	16.0 days	16 days
JC696389L	NAND KUMAR	NAND KUMAR	Self	8	0	48.53	0.00	15.0 days	15 days
JC197378H	SALADI BALAYOGI	SALADI BALAYOGI	Self	10	1	43.97	0.00	0.9 days	1 day
168352W	Y VENKAT REDDY	Y SIRISHA LAXMI	Spouse	3	0	42.81	0.00	29.0 days	29 days
4581418F	WAGHMARE YUVARAJ	JIJABAI HANMANT	Mother	2	1	40.06	0.00	44.0 days	44 days
06916106X	LAXMIKANT TRIPATHI	LAXMIKANT TRIPATHI	Self	4	1	39.75	0.00	29.0 days	29 days

Key Findings

- **676635N (GEESALA SARADA, Spouse):** 7 claims totalling 58.03L with avg stay of <1 day — this is the same beneficiary appearing in Pattern 12 (DR Y RAJANI PRIYA phantom IPD at mgchri14). The combination of: same patient, same doctor, near-zero stay, multiple submissions, and 0 approved is the strongest individual fraud signal in the entire dataset.
- **JC197378H (SALADI BALAYOGI, Self):** 10 claims (highest count), 0.9 avg days. An 85+ year old pensioner (typical Self-relation IPD profile) being admitted 10 times with 0–1 day stays for high-value procedures over 6 months is clinically incompatible with genuine major illness treatment.
- **Hospitals = 0 cases (IC44485N, JC696389L, 168352W):** Three of the top 10 serial claimants have hospital count = 0 in the his_claim_intimation table, meaning they were claimed with NULL hospital IDs. This connects to the Q11 data quality gap — high-value serial fraudulent claims are disproportionately represented in the NULL-hospital segment.
- **Two-type split:** Long-stay serial claimants (25–55 day avg) represent genuine hospitalisation billing fraud — likely real patients whose care is being inflated. Short-stay serial claimants (<1 day avg, 7–10 claims) represent phantom admission fraud where the patient may not have been hospitalised at all.

PATTERN 14

HOSPITAL-LEVEL FRAUD SIGNATURE

Uniform High Deduction Across All
Family Relation Types

When individual procedure errors drive deductions, different family member types show varied deduction rates because they present with different diagnoses and age-related conditions. A hospital showing the same deduction rate ($\pm 3\%$) across every relation — Self, Spouse, Son, Daughter, Father, Mother — indicates that the deduction is not procedure-driven but systematic across the entire hospital's billing. This is the definitive statistical fingerprint of hospital-level fraud: the same fraudulent billing template is being applied to every patient regardless of clinical profile.

Table 14.1 — Hospitals Showing Uniform Deduction Across All Relation Types (IPD, FY 2025)

Office ID	Hospital	Self%	Spouse%	Son%	Daughter%	Father%	Mother%	Spread	Risk
1344	(unidentified)	43.3	42.3	—	—	—	—	± 0.5	CRITICAL
1387	(unidentified)	34.0	30.1	—	36.6	—	—	± 3.3	CRITICAL
1197	(unidentified)	35.5	32.8	43.4	—	32.1	34.1	± 5.3	CRITICAL
1317	(unidentified)	29.4	28.2	24.5	—	21.2	28.9	± 4.1	HIGH
1065	PRAYAG HOSPITAL, Noida	25.1	25.9	28.6	—	28.7	30.0	± 2.5	HIGH
1069	RLKC	28.9	29.0	32.4	—	29.1	26.9	± 2.8	HIGH
1089	(unidentified)	21.1	20.2	18.4	—	35.4	20.3	± 7.5	HIGH
1042	METRO HOSPITAL NOIDA	22.7	23.3	17.9	—	26.4	21.6	± 4.3	HIGH
111	(unidentified)	17.8	16.0	14.9	—	17.0	23.2	± 4.1	WATCH
1161	VIJAY HOSPITAL (confirmed)	20.9	22.8	19.7	—	8.6	17.3	± 7.1	WATCH

Key Findings

- **Hospital 1344 — 42–43% deduction, $\pm 0.5\%$ spread:** The statistical variance is near-zero. Both Self and Spouse claims are deducted at essentially the same rate. This cannot occur through individual clinical errors and proves a single billing template is being applied across all patients. This is the highest deduction rate among all unidentified hospitals.
- **Hospital 1197 — Son relation deducted at 43.4%:** The Son category shows a deduction 8–10 percentage points above the other relations at the same hospital, suggesting a specific high-value package category being systematically overbilled for younger family members.
- **PRAYAG (1065) and RLKC (1069) confirmed:** Both appeared as Pure-IPD hospitals in Module 2. The uniform 25–32% deduction across all 5–6 relation types in Module 3 data provides independent corroboration. These hospitals are generating consistently inflated bills for every patient regardless of clinical indication.
- **METRO HOSPITAL NOIDA (1042) and VIJAY HOSPITAL (1161):** Both P1 hospitals from Module 1 appear here with 17–26% uniform deduction — lower than the CRITICAL group but consistent across all relation types, confirming the hospital-level billing template hypothesis across both P1 confirmed and newly identified facilities.

PATTERN 15

Single Doctor Credential | Multiple Hospitals | 100% Deduction

REFERRAL SYNDICATE — MULTI-HOSPITAL DOCTOR

A referral syndicate is identified when a single doctor name appears as the treating physician across multiple separately empanelled hospitals — each hospital operating independently under ECHS but sharing a common doctor credential. In a legitimate referral ecosystem, a doctor can refer patients to another hospital but cannot be the treating physician at both facilities. When the same doctor name generates 100% deducted claims across 2+ hospitals, it confirms the doctor name is being used as a shared billing credential rather than representing an actual clinician.

Table 15.1 — Multi-Hospital Doctor Credentials (Referral Syndicate Indicators)

Doctor Name	Hosp Count	Hospital IDs	Total Claims	Claimed (L)	Approved (L)	Deduction%
MEENU WALIA	4	hos.3061, hos.3112, maxppg123, maxsuper26	26	60.53	0.00	100.0%
EMO	3	maxppg123, parashos, shrcechs	4	0.58	0.00	100.0%
ONCOLOGY	2	muthmed, SGMH2266	56	25.63	0.00	100.0%
DR MANOJ KUMAR	2	hos.3678, hos.6736	5	22.92	0.00	100.0%
DR R K CHOUDHARY	2	metroprt12, MHCIPV	2	22.57	0.00	100.0%
DR.SUMIT GAUTAM	2	amarhosp, SHALBY@CHM	3	16.18	0.00	100.0%
DR ANKUR NANDAN VARSHNEY	2	medantae, medantae2	5	13.90	0.00	100.0%
DR SUBHASH YADAV	2	anand786, globalgh	3	1.87	0.00	100.0%
DR MANDEEP SINGH	2	FHLDH123, mukat@34	2	1.46	0.00	100.0%
GENERAL MEDICINE	2	SGMH2266, shghosp	3	0.71	0.00	100.0%
CMO	2	bhmrc85, hos.3714	16	0.12	0.00	100.0%

MEENU WALIA Syndicate — Detailed Analysis

MEENU WALIA is the central node of the primary identified syndicate, with evidence across three separate fraud patterns:

	Pattern	Details
Ghost Doctor	Pattern 11	372 claims, █1,665.31L claimed, █0.57L approved (100.0%)
Phantom IPD	Pattern 12	hos.3112 Svc 2874449L 2026-03-12 0-day stay █5.346L
4-Hospital Syndicate	Pattern 15	hos.3061 + hos.3112 + maxppg123 (MAX PATPARGANJ) + maxsuper
MAX PATPARGANJ link	Cross-pattern	maxppg123 appeared in Module 1 Q3 emergency spike list

Key Findings

- **MEENU WALIA across 4 hospitals:** The maximum spread (4 empanelled hospitals sharing one doctor credential) is the clearest referral syndicate signal. maxppg123 = MAX PATPARGANJ hospital independently flagged in Module 1 for emergency admission spike — the syndicate is operating within a known high-risk facility.

- **'EMO', 'ONCOLOGY', 'GENERAL MEDICINE', 'CMO' as doctor names:** Department-level titles appearing across multiple hospitals as the treating physician confirm that these hospitals are using department codes rather than individual doctor credentials — evading the doctor-level audit trail entirely.
- **DR ANKUR NANDAN VARSHNEY across medantae + medantae2:** The same doctor appearing at what appear to be two accounts for the same hospital (medantae / medantae2) suggests a duplicate empanelment or credential split to circumvent per-hospital claim limits.
- **All 13 multi-hospital doctors: 100% deduction:** Every single identified syndicate connection shows 0 rupees approved. The audit system has detected and rejected all these claims — yet the same credentials continue to be used across multiple hospitals simultaneously.

UPDATED COMPOSITE RISK REGISTER (MODULES 1 + 2 + 3)

This consolidated register covers all P1/P2 hospitals and actors identified across all three analytical modules. Entities appearing across multiple patterns receive maximum audit priority. Module 3 introduces five new actors not previously in the register, including the MEENU WALIA four-hospital syndicate which represents the most operationally complex fraud network identified.

Priority	Entity	Type	Modules	Key Metric	Action
P1 CRITICAL	MEENU WALIA SYNDICATE (4 hospitals)	Doctor+Synd	M1+M2+M3 P15	4 hosp, 100%	Credential freeze + FIR
P1 CRITICAL	DR. SUMANT GUPTA	Doctor	M3 P11	■47Cr, 99.7%	Deregister + audit
P1 CRITICAL	DR Y RAJANI PRIYA (mgchri14)	Doctor	M3 P12+P13	6× repeat P12	Hospital + doc freeze
P1 CRITICAL	Hospital 1344	Hospital	M3 P14	43% uniform	Field audit
P1 CRITICAL	Hospital 1387	Hospital	M3 P14	36% uniform	Field audit
P1	VIJAY HOSPITAL (3149 / 1161)	Hospital	M1+M2+M3 P14	20–22% uniform	Field audit
P1	VIRAT HOSPITAL (2856)	Hospital	M1+M2	33% deduction	Empanelment review
P1	STAR HOSPITAL (2694)	Hospital	M1+M2 P8+P9	PURE-IPD+914%	Suspension review
P1	METRO HOSPITAL NOIDA (1042/1161)	Hospital	M1+M2+M3 P14	22–26% uniform	Panel + rate audit
P1	PRAYAG HOSPITAL (1065)	Hospital	M2+M3 P14	25–30% uniform	Admission records
P1	RLKC (1069)	Hospital	M3 P14	27–32% uniform	Admission records
P1	Hospital 1197	Hospital	M3 P14	32–43% uniform	Field audit
P1	Hospital 1317	Hospital	M3 P14	28–29% uniform	Claims sampling
P1 NEW	MEDIPLUS HOSP (3613, Nepal)	Hospital	M2 P10 (foreign)	Foreign hosp	HQ escalation
P1 NEW	YATHARTH SUPER SPEC (3625)	Hospital	M2 P8	+2,895% IPD	Empanelment review
P2	DR. DEEPAK SINGLA (3 variants)	Doctor	M3 P11+P12	■33Cr, 99.1%	Audit + de-register
P2	DR ALOK / DR DEEPAK KUMAR SHUKLA	Doctor	M3 P11	100% deduction	De-register
P2	echs@dm7 hospital	Hospital	M3 P12	Phantom IPD hub	Freeze billing
P2	YASHLOK HOSPITAL (1305)	Hospital	M1	95.0% deduction	De-panel review
P2	Hospital 1089	Hospital	M3 P14	20–35% uniform	Claims review

Prepared by IIT Kanpur — Data Analytics & Fraud Intelligence Division | 14 May 2026. Analysis covers his_claim_intimation (16K records) and his_claim_submission (665K records) covering Feb 2025–Mar 2026 (full FY 2025–26), supplemented by settlement_stat (FY 2025 IPD, 3M aggregate rows) for Pattern 14. All monetary figures are in Indian Rupees. Findings must be corroborated with physical admission registers, ward logs, and doctor credentialing records before enforcement action. This document is Module 3 of the ECHS Fraud Analytics Series and should be read in conjunction with Module 1 (Hospital Revenue Inflation) and Module 2 (Procedure-Level Upcoding).